

CLINICAL DISCHARGE SUMMARY

Patient Name: John Michael Doe
Hospital ID: HSP2026-001245
Age: 58 Years
Gender: Male
Date of Admission: 10-Aug-2026
Date of Discharge: 15-Aug-2026

Chief Complaint

Fever, productive cough, shortness of breath, generalized weakness, and fatigue for five days.

History of Present Illness

58-year-old male admitted with worsening cough associated with yellowish sputum production, intermittent fever with chills, and increasing shortness of breath.

Past Medical History

- Hypertension
- Type 2 Diabetes Mellitus
- Hyperlipidemia
- Former smoker

Vital Signs

Temperature: 38.7°C
Heart Rate: 108 bpm
Respiratory Rate: 24 breaths/min
Blood Pressure: 148/92 mmHg
Oxygen Saturation: 90% on room air

Laboratory Investigations

WBC: $15.6 \times 10^9/L$
CRP: 78 mg/L
Blood Glucose: 186 mg/dL
HbA1c: 8.1%
Creatinine: 1.0 mg/dL

Imaging Findings

Chest X-Ray: Right lower lobe infiltrates suggestive of pneumonia.
CT Chest: Patchy consolidation involving the right lower lobe.

Microbiology

Sputum culture: *Streptococcus pneumoniae*.

Blood cultures negative.

COVID-19 RT-PCR negative.

Hospital Course

Treatment included Ceftriaxone, Azithromycin, oxygen therapy, IV fluids, and glucose monitoring.

Symptoms improved gradually during hospitalization.

Final Diagnosis

1. Community Acquired Pneumonia
2. Type 2 Diabetes Mellitus
3. Hypertension
4. Hyperlipidemia

Medications at Discharge

Amoxicillin-Clavulanate 625 mg TID

Metformin 500 mg BID

Amlodipine 5 mg OD

Atorvastatin 20 mg OD

Follow-Up Instructions

Follow up in 7 days, complete antibiotics, monitor blood glucose, and seek medical attention if symptoms recur.